

Racial discrimination and body appreciation: Testing direct, and, via coping strategies, indirect links in racialised minority adults from the United Kingdom

Nadia Craddock^{a,*}, Jamie Chan^b, Viren Swami^{c,d}

^a Centre for Appearance Research, University of the West of England, Bristol, United Kingdom

^b School of Humanities and Social Science, University of Brighton, Brighton, United Kingdom

^c School of Psychology, Sport, and Sensory Sciences, Anglia Ruskin University, Cambridge, United Kingdom

^d Centre for Psychological Medicine, Perdana University, Kuala Lumpur, Malaysia

ARTICLE INFO

Keywords:

Racial discrimination
Racism
Positive body image
Body appreciation
Coping strategies

ABSTRACT

Racial discrimination is associated with adverse health outcomes, including negative body image, but less is known about the extent to which racial discrimination is associated with positive body image. The aim of this preregistered study was to test the association between racial discrimination and body appreciation (i.e., a facet of positive body image) and to explore whether different coping strategies mediated any direct relationship. An online sample of racialised minority adults living in the United Kingdom ($N = 576$) were asked to complete the Everyday Discrimination Scale, Body Appreciation Scale-2, and the Coping Orientation to Problems Experienced inventory. Preliminary linear modelling showed that seeking emotional social support, positive reinterpretation and growth, turning to religion, and focusing on and venting of emotions were significantly associated with body appreciation. However, findings from subsequent path analysis indicated that racial discrimination was not directly associated with body appreciation. In men only, the direct relationship between racial discrimination and body appreciation was mediated by focus on and venting of emotions. These findings suggest that experiences of racial discrimination may not be associated with positive body image outcomes, at least in racialised minority adults from the United Kingdom. We discuss possible reasons for this, as well as the generally null mediating effects.

1. Introduction

Racial discrimination refers to inequitable, unfair, and unjust treatment of individuals or groups based on their racialised group¹ (Williams et al., 2019). Often conceptualised as the behavioural manifestation of racism, racial discrimination relies on an ideology of racial superiority – oftentimes white supremacy – as well as social structures and interpersonal behaviours associated with dominance, oppression, and prejudice (Kirkinis et al., 2021; Pieterse & Powell, 2016; Reskin, 2012). Accordingly, racial discrimination occurs at multiple levels (i.e., individual/interpersonal, institutional, and cultural; Williams et al., 2019), across multiple domains (e.g., education, employment, housing, criminal justice; Reskin, 2012), and in different forms (e.g., subtle experiences of mistreatment, explicit forms of derogation and intimidation,

rejection or isolation, structural barriers in the forms of lack of opportunities or hostile environments; Carter et al., 2019; Essed, 1991).

There is now a wealth of evidence showing that racial discrimination contributes to health disparities across a range of physical and mental health outcomes (for reviews and meta-analyses, see Carter et al., 2019; Cave et al., 2020; de Freitas et al., 2018; Kirkinis et al., 2021; Paradies et al., 2015). For instance, one recent meta-analysis found robust direct relationships between racial discrimination and poorer mental health ($r = .207$) and poorer physical health ($r = .067$) outcomes (Carter et al., 2019). Much of this work theorises racial discrimination as a chronic social stressor for its targets, who perceive that their individual or collective resources are insufficient to meet the demands of their environment, thereby threatening well-being (Clark et al., 1999; Harrell, 2000). That is, racial discrimination is believed to create a stress reaction in

* Correspondence to: Centre for Appearance Research, University of the West of England, Coldharbour Lane, Bristol BS16 1QY, United Kingdom.

E-mail address: Nadia.Craddock@uwe.ac.uk (N. Craddock).

¹ We use the term racialised group or racialised minority group on the basis that race is a social construct in which identity is assigned based on putative physical similarities within groups (National Academies of Sciences, Engineering and Medicine, 2023).

<https://doi.org/10.1016/j.bodyim.2026.102033>

Received 25 April 2024; Received in revised form 31 July 2025; Accepted 20 January 2026

Available online 27 January 2026

1740-1445/© 2026 The Authors. Published by Elsevier Ltd. This is an open access article under the CC BY license (<http://creativecommons.org/licenses/by/4.0/>).

which the targets of discrimination struggle to cope with the physical and emotional strain of racism (Carter et al., 2019), which in turn has downstream effects on health. Experiences of racial discrimination may also leave individuals with fewer opportunities or resources for making healthy behaviour choices (Pascoe & Richman, 2009).

1.1. Racial discrimination and body image

Consistent with Landor and colleagues' (2024) Sociostructural-Intersectional Body Image (SIBI) framework, factors such as racism are vital to consider for a more comprehensive understanding of body image in racialised minority populations. Indeed, a small body of work has documented an association between greater experiences of racial discrimination and negative body image (e.g., body dissatisfaction, body shame; for a meta-analysis, see Mason et al., 2021). Much of this work is situated in the United States (U.S.) and studies tend to centre a specific racialised minority group and focus on a single gender. For example, Velez and colleagues (2015) reported that more frequent experiences of racial discrimination were associated with greater body shame in Latina women in the U.S. Similarly, Cheng and colleagues found a positive association between experiencing racial discrimination and body dissatisfaction (Cheng et al., 2014), and between experiencing racial discrimination and body shame (Cheng et al., 2017) in two samples of Asian American women. Additionally, in a sample of Asian American men, Cheng et al. (2016) found a positive association between racial discrimination and drive for muscularity. Bhambhani and colleagues (2019) also found that experiencing racial discrimination in sexual and dating contexts was related to higher body dissatisfaction in a sample of U.S. gay/bisexual men of colour.

In explanation of these associations, scholars have generally drawn on the notion that racial discrimination acts as a (minority) stressor that leads to negative outcomes. For instance, Mason and colleagues (2018) proposed a model that links experiences of discrimination with body image concerns via two separate pathways. First, experiences of discrimination heighten the internalisation of unhealthy appearance ideals, possibly as a means of avoiding future discrimination (Mason & Lewis, 2016). Second, experiences of discrimination precipitate maladaptive affect regulation strategies that, in turn, increase states of negative affect that heighten body surveillance and body image concerns (Mason et al., 2019). Although this model was originally developed to explain the incidence of disordered eating in sexual minority women (Mason et al., 2018), evidence of an association between racial discrimination and body dissatisfaction has been interpreted as consistent with this hypothesised model (Mason et al., 2021).

To date, however, less attention has been paid to the possibility that racial discrimination may also affect positive aspects of body image. Positive body image is a multifaceted construct that can be broadly defined as an "overarching love and respect for the body" (Tylka, 2018, p. 9) that includes appreciation of the body and its functions, acceptance of the body despite its imperfections, and protective behaviours such as intuitive eating. Although often inversely correlated with negative body image, positive body image has been conceptualised as a distinct construct that exists on a separate continuum (Tylka & Wood-Barcalow, 2015a). As such, it is possible to simultaneously experience aspects of both negative body image (e.g., weight and shape concerns) and positive body image (e.g., functionality appreciation). Importantly, positive body image is associated with a range of beneficial psychological and behavioural health outcomes including greater life satisfaction, higher self-esteem, self-compassion and emotional wellbeing, and a more robust ability to filter out harmful appearance pressures (Davis et al., 2020; Linardon et al., 2022; Modica & Markley, 2024).

Examining the extent to which racial discrimination may (or may not be) associated with positive body image would help broaden the focus of extant research beyond experiences of psychopathology. Furthermore, it may help to more fully understand the ways in which racism affects the ways racialised minority individuals inhabit and relate to their bodies

(Landor et al., 2024; Ramseyer Winter et al., 2019). Based on available theorising (Mason et al., 2018), we should expect that greater racial discrimination should be associated with lower indices of positive body image because such experiences weaken one's ability or willingness to reject unhealthy (racialised) appearance ideals. Experiences of discrimination may also damage the skills and tools that individuals use to build up resilience in terms of body image experiences or because they limit opportunities to overturn racialised embodiment (Chan et al., 2023).

The limited available evidence supports the idea that experiences of racial discrimination will be associated with lower indices of positive body image. Among young African American women, for example, Dunn and colleagues (2019) reported that experiences of gendered racial microaggressions (i.e., everyday verbal, behavioural, and environmental expressions of systematic oppression based on racialised status and gender) were significantly associated with lower appearance contingent self-worth and lower body appreciation. However, this area of research remains in its infancy and there is a need to more fully assess the extent to which racial discrimination is reliably associated with positive body image outcomes. The first aim of the current study, therefore, was to examine whether racial discrimination is associated with positive body image in a diverse sample of racialised minority adults.

1.2. Coping with racial discrimination

A secondary objective of the present study was to examine whether coping strategies mediate any putative link between racial discrimination and positive body image. This is important because understanding how individuals respond to chronic stressful situations may help scholars and practitioners to identify support strategies that could help alleviate negative health outcomes and promote well-being. In broad outline, the Transactional Model of Stress and Coping (Lazarus & Folkman, 1984) posits that individuals respond to stressful events by enacting coping strategies, which can be defined as "cognitive and behavioural efforts to master, reduce, or tolerate the internal and/or external demands that are created by the stressful transaction" (Folkman, 1984, p. 843). In terms of body image outcomes specifically, a focus on coping strategies is warranted given that existing models suggest that (racial) discrimination heighten the use of maladaptive emotion regulation as a coping strategy, which in turn may lead to body image concerns (Mason et al., 2018; 2021).

Coping strategies and behaviours are categorised in many different ways in the scientific literature, but there is a broad interest in whether (and in what contexts) coping strategies are helpful or harmful in response to different stressors (Skinner et al., 2003). Lazarus and Folkman (1984) distinguished between problem-focused strategies (i.e., eliminating the stressor or reducing its impact) and emotion-focused strategies (i.e., preventing, minimising, or reducing the emotional anguish caused by a stressor). Carver (1997) later added a third broad category of dysfunctional coping strategies, which alludes to maladaptive responses to a stressor. These contrasting coping strategies are typically conceptualised as an approach-avoidance dichotomy (Moos & Holahan, 2003). The available research suggests that racialised minority individuals reactively respond to racism and discrimination using a variety of coping strategies (e.g., Jacob et al., 2023), although they may be more likely to utilise emotion-focused, avoidant strategies (e.g., avoiding thinking about or engaging in the problem and feeling sorry for oneself) to divert attention away from stressful experiences (Vassiliere et al., 2016). Avoidance-based coping, in turn, is associated with increased psychological distress (e.g., Thomas et al., 2008).

Research suggests that associations between racism and wellbeing can be explained by some coping strategies, such as behavioural disengagement and acceptance coping, amongst lesbian, gay, and bisexual people of colour (Huynh & Lee, 2023). However, to date, we are not aware of any research that has specifically examined whether different

coping strategies mediate a relationship between racial discrimination and body image outcomes. Certainly, there is evidence to suggest that the use of avoidant, emotion-focused coping strategies is associated with more negative body image, whereas the use of problem-focused strategies is not significantly associated with body image outcomes (e.g., Koff & Sangani, 1997). Indeed, emotion-focused, avoidant strategies (e.g., emotional eating) appear to be a common response to stressors, at least in women (Smith-Jackson et al., 2011), although this typically results in cycles that contribute to body shame (Bailey et al., 2016). However, how this body of evidence might transfer to experiences of racial discrimination and positive body image outcomes is currently unknown. The most likely possibility is that racial discrimination is associated with emotion-focused strategies, which in turn are associated with lower positive body image.

1.3. The present study

In response to calls for more comprehensive understandings on the body image experiences of racialised minority communities (Landor et al. 2024), the present preregistered study had two related objectives. We first sought to examine the extent to which experiences of racial discrimination are associated with positive body image, which we operationalised through the facet of body appreciation (Tylka & Wood-Barcalow, 2015b). Next, we tested whether coping strategies mediate the putative link between racial discrimination and body appreciation. Using an exploratory approach, we examined a range of lower-order coping strategies (Carver & Scheier, 1994) to identify those significantly associated with body appreciation before constructing a hypothesised mediation model linking racial discrimination and body appreciation via coping strategies.

In an additional departure from the existing research that has focused on the experiences of racialised minority communities in the U.S., the context of the present study is the United Kingdom (U.K.). In the U.K., racism remains a prevalent facet of life for people from racialised minority backgrounds, including in terms of structural effects (e.g., income, employment, education, health, and housing; Finney et al., 2023) and interpersonal effects (e.g., 47 % of racialised minority respondents reported experiencing racial discrimination; Ellingworth et al., 2023). Given this context, it will be useful to examine the extent to which experiences of racial discrimination are both directly and indirectly associated with positive body image outcomes. Here, we hypothesised that experiences of racial discrimination in racialised minority individuals in the U.K. would be associated with lower body appreciation (H1) and that coping strategies would mediate the direct relationship (H2). In terms of H2, we preliminarily hypothesised that emotion-focused, avoidant strategies would emerge as significant mediators.

2. Method

2.1. Participants

A total of 604 respondents provided informed consent and completed the survey. However, we removed 19 respondents who failed one or more attention checks, four who identified as belonging to a mixed racialised group, and five who preferred not to disclose their racialised identity and/or gender identity. This resulted in an analytic sample of 576. Respondents belonging to a mixed racialised group were excluded in the present study due to their unique experiences of racialisation and racial discrimination (e.g., Campion, 2019) and those who did not disclose their gender identity ($n = 2$) were removed due to their small number. Therefore, we retained a dataset consisting of 288 women and 288 men ($M_{\text{age}} = 33.9$ years, $SD = 9.8$), all of whom were of racialised minority status in the context of the U.K. Specifically, 6.9 % identified as Arab or Middle Eastern, 59.7 % identified as Asian, 33.4 % as Black, and 0.3 % as another racialised (minoritised) identity. Additional participant demographics are reported in Table 1.

Table 1

Participant demographics, total and by gender identity, expressed as frequencies (%).

	Total $N = 576$	Women $n = 288$	Men $n = 288$
Racialised Identity			
Arab or Middle Eastern	4.3	1.7	6.9
Asian	58.9	58.0	59.7
East Asian	14.8	15.6	13.9
South Asian	35.1	31.3	38.9
Southeast Asian	9.0	11.1	6.9
Black	36.4	39.9	33.0
Black African	28.6	26.7	30.6
Black Caribbean	7.3	12.2	2.4
Black Other	0.5	1.0	0
Other	0.3	0.3	0.3
Sexual Orientation			
Asexual	2.1	1.4	2.8
Bisexual/Pansexual	5.7	8.3	3.1
Gay/Lesbian	1.4	1.0	1.7
Straight/Heterosexual	88.7	85.8	91.7
Other	0.5	1.0	0
Prefer not to say	1.6	2.4	0.7
Relationship Status			
Divorced	2.1	3.5	0.7
Married	38.9	38.9	28.9
Partnered but not married	18.8	16.7	20.8
Single	38.0	38.2	37.8
Other	1.2	1.7	0.7
Prefer not to say	1.0	1.0	1.0
Highest Educational Qualification			
Secondary schooling (e.g., GCSEs)	2.6	3.8	1.4
Further education (e.g., A-Levels)	10.1	9.4	10.8
Undergraduate degree	52.6	55.2	50.0
Postgraduate degree	28.3	22.9	33.7
Currently in full-time education	3.6	4.9	2.4
Other	2.1	2.4	1.7
Prefer not to say	0.7	1.4	0
Identify as d/Deaf or disabled or have a long-term health condition			
Yes	7.3	9.4	5.2
No	90.6	88.2	93.1
Prefer not to say	2.1	2.4	1.7

2.2. Materials

2.2.1. Body appreciation

To measure a facet of positive body image, participants were asked to complete the Body Appreciation Scale-2 (BAS-2; Tylka & Wood-Barcalow, 2015b). This instrument has 10 items that assess acceptance of one's body, respect and care for one's body, and protection of one's body from unrealistic appearance ideals (sample item: "I respect my body"). All items were rated on a 5-point scale, ranging from 1 (never) to 5 (always). Scores on the BAS-2 has been shown to have a unidimensional factor structure in diverse populations, including in the U.K. (Swami et al., 2023). There is also evidence of strong construct validity (Tylka & Wood-Barcalow, 2015b). An overall score was computed as the mean of all items with higher scores reflecting greater body appreciation. McDonald's ω for BAS-2 scores in the present study was .97 (95 % CI [.96, .97]) in women and .96 (95 % CI [.96, .97]) in men.

2.2.2. Racial discrimination

Experiences of racial discrimination were assessed with the Everyday Discrimination Scale (EDS; Williams et al., 1997), one of the most frequently used instruments for measuring perceived discrimination (Bastos et al., 2010). This is a 9-item self-report scale that measures perceptions of day-to-day discrimination and can be used as a general or

focused measure of discrimination. Adopting the latter approach, participants in this study were asked, “In your day-to-day life, how often have any of the following things happened to you because of your race/ethnicity?” (e.g., being treated with less courtesy), with six response options ranging from never (1) to almost every day (6). Scores on the EDS have been shown to have a unidimensional factor structure and adequate evidence of construct validity (e.g., [Gonzales et al., 2016](#); [Krieger et al., 2005](#)), including in racialised minority communities in the U.K. ([Jaspal et al., 2021](#)). An overall score was computed as the mean of all items, with higher scores indicating more frequent experiences of everyday racial discrimination. McDonald’s ω for the EDS was .93 (95 % *CI* [.92, .94]) in women and .92 (95 % *CI* [.90, .93]) in men.

2.2.3. Coping strategies

Coping strategies were measured using the 60-item Coping Orientation to Problems Experienced (COPE) inventory ([Carver et al., 1989](#)), a theory-driven measure of various coping strategies used to manage difficult situations. The instrument consists of 15 subscales, described in [Table 2](#). Each subscale included four items, with responses ranging from 1 (I usually don’t do this at all) to 4 (I usually do this a lot). Subscale scores were computed as the mean of relevant items. Higher scores on each subscale reflect a greater likelihood of using that type of coping strategy in response to stressful situations. Although there is some debate around the appropriate lower- and higher-order factor structure of the COPE (e.g., [Krägeloh, 2011](#)), the 15 lower-order subscales have generally been found to have adequate composite reliability and provides broad coverage of adaptive and maladaptive coping strategies ([Carver et al., 1989](#)). McDonald’s ω for the COPE subscales in the present study are reported in [Table 2](#).

2.2.4. Demographics

Participants reported their demographic information consisting of gender identity (i.e., *woman*, *man*, *identify my gender in another way*), age, racialised identity, highest educational qualification, marital status, sexual orientation, and whether they identified as a D/deaf or disabled person or whether they had a long-term health condition.

2.3. Procedure

Ethics approval was obtained from the research ethics panel at the first author’s institution (Ref: CHSS.23.08.006) and the study was pre-registered at [redacted]. All data were collected on Prolific, a crowd-working platform that allows researchers to recruit participants ([Palan & Schitter, 2018](#)) between January 3–5, 2024. The project was advertised as a study on “racial discrimination and well-being” and included an estimated duration. Potential respondents on Prolific were pre-screened to ensure that only participants who identified as members of a racialised minority, aged 18 years or over, who were citizens and residents of the U.K., and who were fluent in English were able to complete the survey. Two attention check items were placed between items of the COPE, and one was placed between items of the EDS. Prolific ID codes were examined to ensure that no participant took the survey more than once. After providing digital informed consent, participants provided their demographic details before completing the additional measures, which were presented in a randomised order for each participant. The survey was anonymous and, in exchange for completion, participants were paid £ 1.50.

2.4. Analytic strategy

We first tested for mean differences between women and men across all variables included in the present study and controlled for the false discovery rate (FDR) using the Benjamini-Hochberg procedure ([Benjamini & Hochberg, 1995](#)). Next, we computed bivariate correlations between racial discrimination, the 15 coping strategies, and body appreciation for men and women separately. Additionally, we assessed

Table 2

The COPE inventory subscales with a sample item and composite reliabilities.

Subscale	Sample Item	McDonald’s ω
1 Active coping	I concentrate my efforts on doing something about it	$\omega_{\text{women}} = .77$ (95 % <i>CI</i> [.72, .81]) $\omega_{\text{men}} = .81$ (95 % <i>CI</i> [.77, .85])
2 Planning	I think about how I might best handle the problem	$\omega_{\text{women}} = .83$ (95 % <i>CI</i> [.80, .86]) $\omega_{\text{men}} = .85$ (95 % <i>CI</i> [.82, .88])
3 Suppression of competing activities	I put aside other activities in order to concentrate on this	$\omega_{\text{women}} = .61$ (95 % <i>CI</i> [.52, .68]) $\omega_{\text{men}} = .69$ (95 % <i>CI</i> [.63, .74])
4 Restraint coping	I make sure not to make matters worse by acting too soon	$\omega_{\text{women}} = .64$ (95 % <i>CI</i> [.57, .71]) $\omega_{\text{men}} = .74$ (95 % <i>CI</i> [.68, .78])
5 Seeking instrumental social support	I try to get advice from someone about what to do	$\omega_{\text{women}} = .83$ (95 % <i>CI</i> [.79, .86]) $\omega_{\text{men}} = .87$ (95 % <i>CI</i> [.85, .90])
6 Seeking emotional social support	I talk to someone about how I feel	$\omega_{\text{women}} = .88$ (95 % <i>CI</i> [.86, .90]) $\omega_{\text{men}} = .89$ (95 % <i>CI</i> [.87, .91])
7 Positive reinterpretation & growth	I learn something from the experience	$\omega_{\text{women}} = .79$ (95 % <i>CI</i> [.75, .83]) $\omega_{\text{men}} = .83$ (95 % <i>CI</i> [.80, .86])
8 Acceptance	I accept that this has happened and that it can’t be changed	$\omega_{\text{women}} = .70$ (95 % <i>CI</i> [.63, .75]) $\omega_{\text{men}} = .76$ (95 % <i>CI</i> [.71, .80])
9 Turning to religion	I put my trust in God	$\omega_{\text{women}} = .95$ (95 % <i>CI</i> [.94, .96]) $\omega_{\text{men}} = .94$ (95 % <i>CI</i> [.93, .95])
10 Denial	I act as though it hasn’t even happened	$\omega_{\text{women}} = .74$ (95 % <i>CI</i> [.69, .79]) $\omega_{\text{men}} = .80$ (95 % <i>CI</i> [.76, .83])
11 Behavioural disengagement	I admit to myself that I can’t deal with it, and quit trying	$\omega_{\text{women}} = .76$ (95 % <i>CI</i> [.71, .80]) $\omega_{\text{men}} = .82$ (95 % <i>CI</i> [.78, .85])
12 Mental disengagement	I sleep more than usual	$\omega_{\text{women}} = .49$ (95 % <i>CI</i> [.39, .58]) $\omega_{\text{men}} = .51$ (95 % <i>CI</i> [.40, .59])
13 Focus on & venting of emotions	I get upset and let my emotions out	$\omega_{\text{women}} = .80$ (95 % <i>CI</i> [.75, .83]) $\omega_{\text{men}} = .74$ (95 % <i>CI</i> [.69, .79])
14 Use of humour	I make jokes about it	$\omega_{\text{women}} = .88$ (95 % <i>CI</i> [.85, .90]) $\omega_{\text{men}} = .89$ (95 % <i>CI</i> [.87, .91])
15 Alcohol/drug use	I use alcohol or drugs to help me get through it	$\omega_{\text{women}} = .91$ (95 % <i>CI</i> [.90, .93]) $\omega_{\text{men}} = .93$ (95 % <i>CI</i> [.92, .94])

associations between the coping strategies and body appreciation using linear model analyses, conducted using the *robust* package ([Wang et al., 2013](#)) in R ([R Development Core Team, 2021](#)), which deals with non-normality. To identify which of the different coping strategies were associated with body appreciation, we constructed a linear model where the 15 coping strategies were modelled simultaneously as predictor variables and body appreciation was modelled as the criterion variable. Only coping strategies that were significantly associated with body appreciation were retained as mediators in subsequent analyses.

Based on the previous analyses, we constructed a hypothesised model linking racial discrimination and body appreciation via retained

coping strategies (see Fig. 1). To test the fit of this model to the data, we used path analysis conducted using the *lavaan* package (Rosseel, 2012) in R. As there were no missing data, Maximum Likelihood with Robust standard errors (MLR) was used to account for non-normality in the data. We assessed the fit of the model using the model chi-square value, the comparative fit index (CFI; with values close to or $\geq .95$ indicating good fit), the Stieger-Lind root mean square error of approximation (RMSEA) and its 90 % CI (values close to .06 considered to be indicative of good fit and up to .08 indicative of adequate fit) and the standardised root mean square residual (SRMR; values $< .09$ indicative of good fit) (Hu & Bentler, 1999; Swami & Barron, 2019). If the model needed improving, we aimed to add or release constraints for particular paths based on modification indices (with values > 3.84 suggesting potential improvements in model fit).

Based on the final path analytic model (with adequate fit), we fitted a constrained model and an unconstrained model to assess model invariance across women and men. We then compared the fit of the constrained model to that of the unconstrained model using a likelihood ratio test (i.e., chi-square values) and the comparative fit index, with decreases in CFI $> .01$ indicating worse model fit (Cheung & Rensvold, 2002). A significantly worse fitting model would indicate invariance between men and women. Reactive Monte Carlo simulations in Mplus v. 8.11 (Muthén & Muthén, 2024) showed that our sample size was adequate for detecting the hypothesised direct and mediation effects. The analysis was based on a significance level of $p < .05$, statistical power at .95, and an effect size of .10. The sample size in this study exceeded the minimum required threshold.

3. Results

3.1. Preliminary analyses

Descriptive statistics and FDR-corrected independent-samples *t*-tests are reported in Table 3. Women had significantly higher scores on the COPE subscales of social emotional support-seeking, mental disengagement, and focus on and venting of emotions than men, whereas men scored significantly higher in the use of humour and alcohol-drug use subscales than women. No significant gender differences were found in racial discrimination and body appreciation scores.

Bivariate correlations between racial discrimination, the 15 coping strategies, and body appreciation are reported in Table 4. For both women and men, racial discrimination was not significantly associated with body appreciation. In men, racial discrimination was significantly associated with higher scores on all coping strategies except active coping, planning, positive reinterpretation and growth, and acceptance. Men's body appreciation was also significantly linked with all coping strategies except denial, mental disengagement, and focus on and venting of emotions. For the coping strategies that were significantly correlated with men's body appreciation, higher scores were linked with greater body appreciation, except for behavioural disengagement and

Table 3

Descriptive statistics for all variables included in the present study and reporting of between-group differences between women and men.

	Women		Men		<i>t</i>	<i>p</i>	<i>d</i>
	<i>M</i>	<i>SD</i>	<i>M</i>	<i>SD</i>			
BAS-2	3.55	1.01	3.67	.92	-1.48	.264	.12
EDS	2.34	.95	2.37	.89	-.32	.797	.03
AC	11.06	2.50	10.85	2.72	.96	.525	.08
Planning	11.43	2.74	11.29	2.99	.16	.870	.01
SoCA	9.95	2.21	10.25	2.36	-1.53	.264	.13
RC	9.93	2.26	10.09	2.52	-.77	.615	.06
SSS-I	10.19	3.09	9.67	3.22	1.97	.142	.16
SSS-E	10.24	3.28	8.83	3.24	5.21	.009	.43
PRG	11.32	2.67	11.19	2.86	.54	.669	.05
Acceptance	10.62	2.44	10.92	2.55	-1.44	.619	.12
TTR	10.50	4.54	9.86	4.38	1.74	.194	.15
Denial	6.34	2.37	6.56	2.51	-1.11	.459	.09
BD	6.80	2.42	6.65	2.53	.72	.615	.06
MD	9.82	2.46	9.25	2.25	2.88	.017	.24
FoVoE	10.24	2.88	8.27	2.60	8.65	.009	.72
UoH	7.61	2.85	8.21	3.15	-2.40	.068	.20
ADU	4.78	1.91	5.38	2.60	3.14	.011	.26

Note: BAS-2 = Body Appreciation Scale-2, EDS = Everyday Discrimination Scale, AC = Active coping, SoCA = Suppression of competing activities, RC = Restraint, SSS-I = Seeking social support – instrumental, SSS-E = Seeking social support – emotional, TTR = Turning to religion, BD = Behavioural disengagement, MD = mental disengagement, FoVoE = Focus on and venting of emotions, UoH = Use of humour, ADU = Alcohol-drug use.

alcohol-drug use – these were linked with lower body appreciation. For women, racial discrimination was significantly associated with greater restraint coping, denial, use of humour and alcohol-drug use. Women's body appreciation was significantly associated with all coping strategies, except denial and mental disengagement. For the coping strategies that were significantly correlated with women's body appreciation, higher scores were linked with greater body appreciation, except behavioural disengagement, focus on and venting of emotions, and alcohol-drug use – these strategies were linked with lower body appreciation.

The associations between the different coping strategies and body appreciation were further tested in a linear model (see Table 5). Notably, only the seeking emotional social support ($\beta = 3.01, p = .003$), positive reinterpretation and growth ($\beta = 3.44, p < .001$), turning to religion ($\beta = 3.69, p < .001$), and focus on and venting of emotions ($\beta = -4.15, p < .001$) subscales were significantly associated with body appreciation. These were modelled as mediators in subsequent analyses.

3.2. Preregistered hypothesis-testing

The hypothesised model demonstrated poor fit to the data, $\chi^2(6, N = 576) = 368.39, p < .001$; CFI = .41; RMSEA = .32 (90 % CI [.30, .35]); SRMR = .19 (see Fig. 1). Based on modification indices, paths

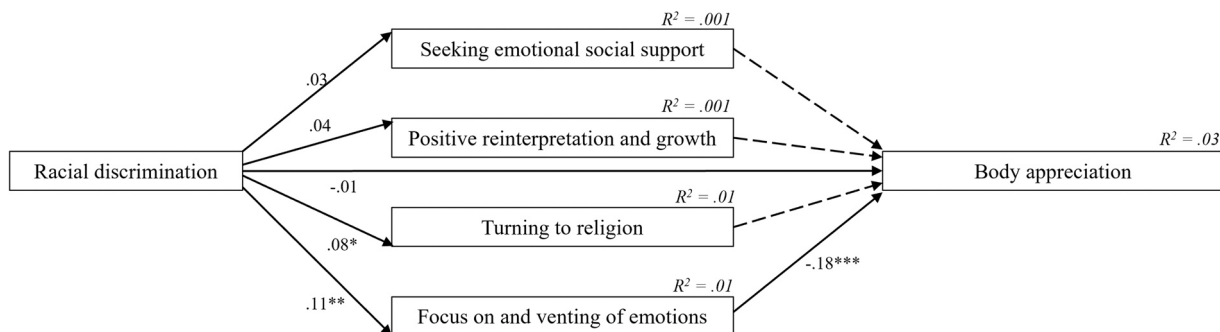


Fig. 1. Hypothesised and Final Path Analytic Model. Note. Dotted lines indicate pathways that were included in the hypothesised model but removed from the final model. Covariation paths omitted for clarity.

Table 4

Zero order correlations between all variables included in the present study, with women in the top diagonal and men in the bottom diagonal.

	1	2	3	4	5	6	7	8	9	10	11	12	13	14	15	16	17
1. BAS-2																	
2. EDS	-.03																
3. AC	.49**	.05															
4. Planning	.48**	.01	.82**														
5. SoCA	.40**	.18**	.71**	.71**													
6. RC	.26**	.21**	.40**	.41**	.44**												
7. SSS-I	.41**	.13*	.59**	.55**	.57**	.35**											
8. SSS-E	.40**	.12*	.48**	.43**	.50**	.33**	.82**										
9. PRG	.54**	.09	.70**	.72**	.60**	.39**	.54**	.43**									
10. Acceptance	.23**	.06	.30**	.27**	.29**	.37**	.14*	.15*	.37**								
11. TTR	.46**	.16**	.37**	.37**	.42**	.34**	.42**	.41**	.48**	.13*							
12. Denial	.05	.31**	.02	-.04	.19**	.19**	.23**	.25**	.14*	.02	.30**						
13. BD	-.25**	.26**	-.16**	-.19**	.02	.17**	.02	.07	-.11	.05	.01	.49**					
14. MD	-.04	.22**	.11	.01	.20**	.28**	.22**	.26**	.17**	.16**	.17**	.39**	.37**				
15. FoVoE	-.10	.23**	.16**	.12*	.29**	.17**	.40**	.41**	.08	.01	.13*	.35**	.44**	.40**			
16. UoH	.15**	.16**	.21**	.15*	.15**	.16**	.16**	.15**	.36**	.20**	.10	.24**	.19**	.25**	.04		
17. ADU	-.16**	.36**	-.02	.03	.07	.12*	.12*	.10	.02	.07	-.03	.37**	.44**	.23**	.30**	.17**	

Note: BAS-2 = Body Appreciation Scale-2, EDS = Everyday Discrimination Scale, AC = Active coping, SoCA = Suppression of competing activities, RC = Restraint Control, SSS-I = Seeking social support – instrumental, SSS-E = Seeking social support – emotional, TTR = Turning to religion, BD = Behavioural disengagement, MD = mental disengagement, FoVoE = Focus on and venting of emotions, UoH = Use of humour, ADU = Alcohol-drug use.

Table 5

Full coefficients for the linear model.

Predictors of body appreciation	B	β	SE	p
Active coping	.05	1.86	.03	.064
Planning	.01	.39	.03	.696
Suppression of competing activities	-.01	-.38	.03	.707
Restraint coping	.01	.74	.02	.460
Seeking social support – instrumental	-.01	-.39	.02	.696
Seeking social support – emotional	.06	3.01	.02	.003
Positive reinterpretation and growth	.08	3.44	.02	< .001
Acceptance	.02	.87	.02	.386
Turning to religion	.04	3.69	.01	< .001
Denial	.04	1.90	.02	.058
Behavioural disengagement	-.04	-1.76	.02	.079
Mental disengagement	-.04	-1.81	.02	.072
Focus on and venting of emotions	-.07	-4.15	.02	< .001
Use of humour	.01	.57	.01	.640
Alcohol-drug use	-.04	-1.89	.02	.059

were removed between body appreciation and: (i) seeking emotional social support; (ii) positive reinterpretation and growth; and (iii) turning to religion. Further, covariation paths were added between focus on and venting of emotions and seeking emotional social support, and between positive reinterpretation and growth and turning to religion. This improved the model fit and the final path model had adequate fit, $\chi^2(2, N = 576) = 4.18, p = .123$; CFI = .996; RMSEA = .04 (90 % CI [.01.10]); SRMR = .02 (see Fig. 1).

Subsequent multigroup path analysis suggested that constraining the intercepts and paths between variables resulted in significantly worse

model fit in the constrained model comparison to the unconstrained model ($\Delta\chi^2 = 91.21, p < .001$; $\Delta CFI = .13$). This suggests that the relationships between racial discrimination, coping strategies, and body appreciation were different for men and women. For women, only focus on and venting of emotions was significantly and negatively associated with body appreciation ($\beta = -.17, p < .001$). No other significant direct or indirect relationships were found in women (see Table 6).

For men, racial discrimination was significantly associated with seeking emotional social support ($\beta = .12, p = .051$), turning to religion ($\beta = .16, p = .006$), and focus on and venting of emotions ($\beta = .23, p < .001$), but not positive reinterpretation and growth ($\beta = .09, p = .109$). Focus on and venting of emotions was negatively associated with body appreciation ($\beta = -.15, p < .001$). Racial discrimination was not directly associated with body appreciation but was indirectly associated with body appreciation through focus on and venting of emotions ($\beta = -.03, p = .012$) (see Table 6).

3.3. Unregistered testing

We additionally conducted three unregistered robustness tests. First, to account for any potential deflation of experiences of racial discrimination due to the current method of scoring the EDS, we coded the EDS to reflect chronicity. In this method, EDS responses are coded to reflect the total number of reported discrimination experiences, standardised on the total number of days per year (for details, see Michaels et al., 2019). Findings suggested that the direct relationship between body appreciation and the chronicity-coded EDS remained non-significant ($\beta = .05, p = .961$). Second, to account for demographic variability, we ran

Table 6

Coefficients for women and men separately.

	Women					Men				
	B	β	SE	p	R ²	B	β	SE	p	R ²
Direct effects										
Everyday discrimination on										
Seeking social support – emotional	-.15	-.03	.06	.489	.002	.44	.12*	.06	.051	.02
Positive reinterpretation and growth	-.06	-.02	.06	.724	< .001	.29	.09	.06	.109	.01
Turning to religion	.07	.02	.06	.793	< .001	.89	.16**	.06	.006	.03
Focus on and venting of emotions	.09	.03	.06	.635	.001	.66	.23***	.06	< .001	.05
Body appreciation from										
Everyday discrimination	-.03	-.03	.06	.675	-	.01	.01	.06	.935	-
Focus on and venting of emotions	-.06	-.17***	.05	.001	.03	-.05	-.15***	.05	.001	.02
Indirect effects										
Everyday discrimination on body appreciation via focus on and venting of emotions	-.01	-.01	.01	.63	-	-.04	-.03*	.01	.012	-

a version of the final model with ethnicity, sexual orientation, marital status, education and health condition as covariates. The model fit remained stable with acceptable CFI, RMSEA and SRMR values (CFI = .99, RMSEA = .07; SRMR = .02), although the chi-square statistic became significant ($p = .02$).

Third, considering that most of the 15 COPE subscales were correlated with one another, we further modelled higher-order factor scores of the COPE as latent variables in the path analyses to reduce multicollinearity. These higher-order factors were problem-focused coping (e.g., active coping, planning, suppression of competing activities, restraint coping and seeking social support – instrumental), emotion-focused coping (e.g., seeking social support – emotional, positive reinterpretation and growth, acceptance, turning to religion and denial), ‘less useful’ (e.g., behavioural disengagement, mental disengagement and focus on and venting of emotions) and hedonistic escapism (e.g., use of humour and alcohol-drug use; Litman, 2006). Racial discrimination was only significantly associated with hedonistic coping ($\beta = .43$, $p < .001$); emotion-focused ($\beta = .58$, $p < .001$) and ‘less useful’ coping ($\beta = -.33$, $p < .001$) were significantly associated with body appreciation in the path analysis, $\chi^2(71, N = 576) = 217.12$, $p < .001$; CFI = .96; RMSEA = .06 (90 % CI [.05, .07]); SRMR = .07. No significant direct relationships were found between racial discrimination or body appreciation, and the model was invariant between genders ($\Delta\chi^2 = 77.23$, $p < .001$; Δ CFI = .02; see Figure S1 and Table S1 in Supplementary Materials).

4. Discussion

This preregistered study tested whether racial discrimination was significantly associated with body appreciation among a sample of racialised minority participants in the U.K. (H1), and whether coping strategies mediated this relationship (H2). Contrary to H1, racial discrimination was not significantly and directly associated with body appreciation. Further, our results only marginally supported H2. While preliminary linear modelling showed that several coping strategies were significantly associated with body appreciation, just one strategy – focus on and venting of emotions – mediated a relationship between racial discrimination and body appreciation. Notably, however, multigroup path analysis indicated that this result was only significant in men. Overall, these findings suggest that racial discrimination is neither directly nor, with one minor exception, indirectly associated with body appreciation via common coping strategies.

4.1. Reflections on our H1 findings

Our findings showed that racial discrimination was not significantly associated with a central facet of positive body image, namely body appreciation. This finding is inconsistent with prior theoretical modelling suggesting that experiences of racial discrimination should be associated with body image outcomes (Mason et al., 2018; 2021). It also stands in contrast to the limited research that has centred facets of positive body image; specifically, it contrasts with the findings of Dunn and colleagues (2019), who showed that gendered racial micro-aggression stress was both directly and indirectly (via appearance-contingent self-worth) associated with lower body appreciation in a sample of African American women.

One explanation for our null association between racial discrimination and body appreciation is that reported experiences of racial discrimination were relatively infrequent in our sample. Mean responses were just over 2.0 (on a 6-point scale), suggesting that our sample did not appear to be chronically affected by racial discrimination. Thus, any putative relationship between racial discrimination and body appreciation may have been affected by floor effects. Relatedly, a meta-analytic review (Schmitt et al., 2014) found that associations between perceived discrimination (including racial discrimination) and positive outcomes (e.g., self-esteem, life satisfaction, positive affect) were generally weaker than for negative outcomes (e.g., depression, anxiety, psychological

distress). Therefore, it is possible that examining a relationship between racial discrimination and body appreciation (a positive outcome) may be more vulnerable to floor effects than a relationship between racial discrimination and facets of negative body image, such as body dissatisfaction.

Another possible explanation pertains to our chosen measurement of racial discrimination. The EDS is a brief, 9-item measure that measures the frequency of perceived experiences of mistreatment and negative stereotyping. We selected this measure based on its widespread use, which allows for comparison across studies, as well as its brevity to minimise participant burden. However, the EDS does not capture the severity or intensity of racial discrimination, or the associated stress felt by individuals. A more comprehensive measure – such as the Schedule of Racist Events (Landrine & Klonoff, 1996), which captures the frequency as well as the extent to which experiences of racial discrimination are felt as stressful – may have garnered different results. Additionally, the EDS does not include the impact of anticipated racism, which could include racism at the structural or systemic level, as well as interpersonal racism. Capturing the stress felt by individuals due to racial discrimination and/or anticipated racism, which itself may serve as a more chronic stressor, may be more closely related to how individuals appreciate, respect, and feel comfortable in their bodies.

Finally, taking our results at face value, it is possible that racial discrimination – irrespective of frequency, severity, or associated stress – is not associated with body appreciation. Our hypothesising in the present study was largely driven by results obtained with indices of negative body image, but negative and positive body image do not lay on the same continuum (Tylka & Wood-Barcalow, 2015a). Therefore, it is possible for a construct to be related to negative body image but not to positive body image – as may be the case for racial discrimination – and *vice versa*. Individuals may feel greater body dissatisfaction (i.e., are less content with their appearance) in response to racial discrimination because such experiences are often prompted by appearance-related cues, such as skin colour and racialised facial features. But, by contrast, racial discrimination may not as readily affect body appreciation, which is more holistic in nature and reflects a deeper connection to one's body (Tylka & Wood-Barcalow, 2015a).

4.2. Reflections on our H2 findings

Turning to the mediating role of coping, in general, we found that coping strategies did not mediate the link between racial discrimination and body appreciation, either at subscale or latent factor levels. The most likely reason for this is because there was no direct relationship between racial discrimination and body appreciation. However, we did find that, in men only, focusing on and venting of emotions mediated the relationship between racial discrimination and body appreciation.

In our preliminary linear modelling, some coping strategies were significantly associated with body appreciation. Specifically, we found that seeking emotional social support, positive reinterpretation and growth, turning to religion, and focus on and venting of emotions were significantly associated with body appreciation. Body appreciation has previously been positively associated with proactive coping (Avalos et al., 2005) and engagement with religion and/or spirituality (Tiggemann & Hage, 2019) in samples of North American and Australian women, respectively. Therefore, our results support and extend these earlier findings to men, as well as a new national context. Further, qualitative research has pointed to emotional social support as a factor that helps to foster a positive body image (Wood-Barcalow et al., 2010). The negative association between focusing on and venting emotions and body appreciation is more novel, though it is consistent with research on psychological well-being more broadly. Focusing on and venting emotions can be defined as monitoring and expressing distressing emotions. While it could be cathartic to release negative emotions, it may lead an individual to dwell on negative experiences. Thus, focusing on and venting emotions can be considered a less proactive form of coping and

can exacerbate the adverse effects of racial discrimination on an individual's well-being (Brondolo et al., 2005; Kaholokula et al., 2017).

Beyond these direct associations between coping strategies and body appreciation, however, our results generally showed that coping strategies did not mediate any relationship between racial discrimination and body appreciation – with one exception. Specifically, in men only, focusing on and venting of emotions mediated the relationship between racial discrimination and body appreciation. Findings from a previous meta-analysis suggested that gender differences in coping were influenced by differences in the types of stressful situations (Tamres et al., 2002); in particular, men were more likely than women to use venting as a coping strategy when dealing with social stressors. Interpersonal racial discrimination is often positioned as a form of chronic social stress (Clark et al., 1999). Therefore, it is plausible that this shaped men's increased focus on and venting of emotions as a coping strategy, which in turn was associated with body appreciation outcomes.

It is important to note that the COPE strategies were not tailored to focus on coping strategies in relation to racial discrimination; rather, items pertain to coping in relation to any stressful situation. Conversely, resistance and empowerment against racism can be positioned as a form of coping in response to racial discrimination specifically (Suyemoto et al., 2022). In a sample of racialised minority women and men from the U.K., Chan and colleagues (2023) reported that facets of resistance and empowerment against racism (i.e., leadership for resistance against racism and interpersonal confrontation of racism) were significantly associated with greater body appreciation and body acceptance by others.

4.3. Limitations and future directions

There are several important limitations that warrant discussion, which may be addressed in subsequent studies. First, as this study was cross-sectional in design, the direction of association between variables should be understood with caution. Although we developed a hypothesised model that conforms to current theorising, any causal inference should be treated with caution. Additionally, studies show that cumulative exposure to racial discrimination has incremental negative long-term effects on the mental health of racialised minority people in the U.K. (Wallace et al., 2016). Therefore, our findings are limited by considering associations between racial discrimination and positive body image at a single point in time. As our cross-sectional findings present mostly null results, we suggest researchers consider examining longitudinal relationships between racial discrimination and multiple aspects of both positive and negative body image.

Second, our sample is not representative, meaning that generalisations cannot be readily made. Further, it was not possible to examine potential differences by racialised group due to insufficient sample size in each subgroup. Such analyses may have yielded more nuanced results. For example, data from a large-scale, U.K. based study showed that both cross-sectional and longitudinal associations between racial discrimination and psychological and physical health outcomes are not consistent across racialised groups (Hackett et al., 2020). Similarly, in their systematic review based on predominantly U.S. data, Paradies and colleagues (2015) highlighted that racialised group significantly moderated associations between racial discrimination and various health outcomes. Therefore, it is possible that the association between racial discrimination and body appreciation may vary depending on racialised identity.

Third, we did not assess skin shade and so we also did not include skin shade as a covariate in our analysis. Prior research indicates that Black, Asian and other racialised minority adults in the U.K. with darker skin report greater exposure to racial discrimination than their racialised group peers with light skin (Craddock, Phoenix et al., 2023). Studies also show that skin shade is often relevant to body image (e.g., body dissatisfaction and body esteem) for racialised minorities in the U.K. (Chan & Hurst, 2022; Craddock Gentili et al., 2023). Therefore,

including skin shade would have been a potentially valuable addition and could be explored in future work on racial discrimination and positive body image.

4.4. Conclusion

In conclusion, our findings suggest that experiences of racial discrimination may not be associated with body appreciation in racialised minority participants from the U.K., and that coping strategies do not effectively mediate any direct relationship. In view of this finding, one useful avenue for future research would be to replicate the present work but include indices of both negative and positive body image, which would provide an opportunity to test the extent to which our findings are anomalous. In a similar vein, it may be useful to consider other facets of positive body image that may be more directly impacted by racial discrimination, such as body pride (e.g., see Le et al., 2022). More generally, it may also be important to replicate the present finding in other national contexts, to help determine whether our findings are specific to the U.K. or more broadly generalisable. Certainly, there is scope to more fully consider the extent to which racial discrimination may – or may not be – associated with indices of positive body image in diverse communities.

CRediT authorship contribution statement

Viren Swami: Writing – review & editing, Project administration, Methodology, Conceptualization. **Jamie Chan:** Writing – review & editing, Writing – original draft, Methodology, Formal analysis, Conceptualization. **Nadia Craddock:** Writing – review & editing, Writing – original draft, Resources, Project administration, Methodology, Conceptualization.

Declaration of Competing Interest

The authors declare that they have no known competing financial interests or personal relationships that could have appeared to influence the work reported in this paper.

Appendix A. Supporting information

Supplementary data associated with this article can be found in the online version at doi:10.1016/j.bodyim.2026.102033.

Data availability

Data will be made available on request.

References

- Avalos, L., Tylka, T. L., & Wood-Barcalow, N. (2005). The body appreciation scale: Development and psychometric evaluation. *Body image*, 2(3), 285–297. <https://doi.org/10.1016/j.bodyim.2005.06.002>
- Bailey, K. A., Lamarch, L., Gammage, K. L., & Sullivan, P. J. (2016). Self-objectification and the use of body image coping strategies: The role of shame in highly physically active women. *The American Journal of Psychology*, 129(1), 81–90. <https://doi.org/10.5406/amerjpsyc.129.10081>
- Bastos, J. L., Celeste, R. K., Faerstein, E., & Barros, A. J. (2010). Racial discrimination and health: A systematic review of scales with a focus on their psychometric properties. *Social Science Medicine*, 70(7), 1091–1099. <https://doi.org/10.1016/j.socscimed.2009.12.020>
- Benjamini, Y., & Hochberg, Y. (1995). Controlling the false discovery rate: A practical and powerful approach to multiple testing. *Journal of the Royal Statistical Society Series B*, 57(1), 289–300. <https://doi.org/10.1111/j.2517-6161.1995.tb02031.x>
- Bhambhani, Y., Flynn, M. K., Kellum, K. K., & Wilson, K. G. (2019). Examining sexual racism and body dissatisfaction among men of color who have sex with men: The moderating role of body image inflexibility. *Body Image*, 28, 142–148. <https://doi.org/10.1016/j.bodyim.2019.01.007>
- Brondolo, E., Thompson, S., Brady, N., Appel, R., Cassells, A., Tobin, J. N., & Sweeney, M. (2005). The relationship of racism to appraisals and coping in a community sample. *Ethnicity Disease*, 15, 14–19. (<https://www.jstor.org/stable/4866721>).

- Campion, K. (2019). You think you're Black?" Exploring Black mixed-race experiences of Black rejection. *Ethnic and Racial Studies*, 42(16), 196–213. <https://doi.org/10.1080/01419870.2019.1642503>
- Carter, R. T., Johnson, V. E., Kirkinis, K., Roberson, K., Muchow, C., & Galgay, C. (2019). A meta-analytic review of racial discrimination: Relationships to health and culture. *Race and Social Problems*, 11, 15–32. <https://doi.org/10.1007/s12552-018-9256-y>
- Carver, C. S. (1997). You want to measure coping but your protocol's too long: Consider the brief COPE. *International Journal of Behavioral Medicine*, 4, 92–100. https://doi.org/10.1207/s15327558ijbm0401_6
- Carver, C. S., Scheier, M. F., & Weintraub, J. K. (1989). Assessing coping strategies: A theoretically based approach. *Journal of Personality and Social Psychology*, 56(2), 267–283. <https://doi.org/10.1037/0022-3514.56.2.267>
- Carver, C. S., & Scheier, M. F. (1994). Situational coping and coping dispositions in a stressful transaction. *Journal of Personality and Social Psychology*, 66(1), 184–195. <https://doi.org/10.1037/0022-3514.66.1.184>
- Cave, L., Cooper, M. N., Zubrick, S. R., & Shepherd, C. C. J. (2020). Racial discrimination and child and adolescent health in longitudinal studies: A systematic review. *Social Science Medicine*, 250, Article 112864. <https://doi.org/10.1016/j.socscimed.2020.112864>
- Chan, J., Craddock, N., & Swami, V. (2023). Resistance and empowerment against racism: A preregistered study of associations with body appreciation and body acceptance by others in racialised minority adults in the United Kingdom. *Body Image*, 46, 1–8. <https://doi.org/10.1016/j.bodyim.2023.04.010>
- Chan, J., & Hurst, M. (2022). South Asian women in the United Kingdom: The role of skin colour dissatisfaction in acculturation experiences and body dissatisfaction. *Body Image*, 42, 413–418. <https://doi.org/10.1016/j.bodyim.2022.07.007>
- Cheng, H. L. (2014). Disordered eating among Asian/Asian American women: Racial and cultural factors as correlates. *The Counseling Psychologist*, 42(6), 821–851. <https://doi.org/10.1177/0011000014535472>
- Cheng, H.-L., McDermott, R. C., Wong, Y. J., & La, S. (2016). Drive for muscularity in Asian American men: Sociocultural and racial/ethnic factors as correlates. *Psychology of Men Masculinity*, 17(3), 215–227. <https://doi.org/10.1037/men0000019>
- Cheng, H. L., Tran, A. G., Miyake, E. R., & Kim, H. Y. (2017). Disordered eating among Asian American college women: A racially expanded model of objectification theory. *Journal of Counseling Psychology*, 64(2), 179–191. <https://doi.org/10.1037/cou0000195>
- Cheung, G. W., & Rensvold, R. B. (2002). Evaluating goodness-of-fit indexes for testing measurement invariance. *Structural Equation Modeling*, 9(2), 233–255. https://doi.org/10.1207/s15328007sem0902_5
- Clark, R., Anderson, N. B., Clark, V. R., & Williams, D. R. (1999). Racism as a stressor for African Americans: A biopsychosocial model. *American Psychologist*, 54(10), 805–816. <https://psycnet.apa.org/doi/10.1037/0003-066X.54.10.805>
- Craddock, N., Gentili, C., Phoenix, A., White, P., Diedrichs, P. C., & Barlow, F. K. (2023). Investigating the role of perceived ingroup and outgroup colourism on body image and wellbeing among Black, Asian, and other racialised/ethnic minority groups living in the UK. *Body Image*, 46, 246–255. <https://doi.org/10.1016/j.bodyim.2023.06.010>
- Craddock, N., Phoenix, A., White, P., Gentili, C., Diedrichs, P. C., & Barlow, F. (2023). Understanding colourism in the UK: Development and assessment of the everyday colourism scale. *Ethnic and Racial Studies*, 46(10), 2242–2277. <https://doi.org/10.1080/01419870.2022.2149275>
- Davis, L. L., Fowler, S. A., Best, L. A., & Both, L. E. (2020). The role of body image in the prediction of life satisfaction and flourishing in men and women. *Journal of Happiness Studies*, 21(2). <https://doi.org/10.1007/s10902-019-00093-y>
- de Freitas, D. F., Fernandes-Jesus, M., Ferreira, P. D., Coimbra, S., Teixeira, P. M., de Moura, A., Gato, J., Marques, S. C., & Fontaine, A. M. (2018). Psychological correlates of perceived ethnic discrimination in Europe: A meta-analysis. *Psychology of Violence*, 8(6), 712–725. <https://doi.org/10.1037/vio0000215>
- Dunn, C. E., Hood, K. B., & Owens, B. D. (2019). Loving myself through thick and thin: Appearance contingent self-worth, gendered racial microaggressions and African American women's body appreciation. *Body Image*, 30, 121–126. <https://doi.org/10.1016/j.bodyim.2019.06.003>
- Ellingworth, D., Bécares, L., Štašná, M., & Nazroo, J. (2023). Racism and racial discrimination. In N. Finney, J. Nazroo, L. Bécares, D. Kapadia, & N. Shlomo (Eds.), *Racism and ethnic inequality in a time of crisis* (pp. 54–77). Bristol, UK: Policy Press. <https://doi.org/10.51952/9781447368861.ch004>
- Essed, P. (1991). *Understanding everyday racism: An interdisciplinary theory*, 2. London, UK: Sage.
- Finney, N., Nazroo, J., Bécares, L., Kapadia, D., & Shlomo, N. (Eds.). (2023). *Racism and ethnic inequality in a time of crisis*. Bristol, UK: Policy Press. <https://doi.org/10.56687/9781447368861>
- Folkman, S. (1984). Personal control and stress and coping processes: A theoretical analysis. *Journal of Personality and Social Psychology*, 46(4), 839–852. <https://doi.org/10.1037/0022-3514.46.4.839>
- Gonzales, K. L., Noonan, C., Goins, R. T., Henderson, W. G., Beals, J., Manson, S. M., Acton, K. J., & Roubideaux, Y. (2016). Assessing the everyday discrimination scale among American Indians and Alaska Natives. *Psychological Assessment*, 28(1), 51–58. <https://doi.org/10.1037/a0039337>
- Hackett, R. A., Ronaldson, A., Bhui, K., Steptoe, A., & Jackson, S. E. (2020). Racial discrimination and health: a prospective study of ethnic minorities in the United Kingdom. *BMC Public Health*, 20, 1–13. <https://doi.org/10.1186/s12889-020-09792-1>
- Harrell, S. P. (2000). A multidimensional conceptualization of racism-related stress: Implications for the well-being of people of color. *American Journal of Orthopsychiatry*, 70(1), 42–57. <https://doi.org/10.1037/h0087722>
- Hu, L. T., & Bentler, P. M. (1999). Cutoff criteria for fit indexes in covariance structure analysis: Conventional criteria versus new alternatives. *Structural Equation Modeling*, 6(1), 1–55. <https://doi.org/10.1080/10705519909540118>
- Huynh, K. D., & Lee, D. L. (2023). Emotion-focused coping strategies as mediators of the discrimination–mental health association among LGB POC. *Psychology of Sexual Orientation and Gender Diversity*, 10(3), 413–428. <https://doi.org/10.1037/sgd0000562>
- Jacob, G., Faber, S. C., Faber, N., Bartlett, A., Ouimet, A. J., & Williams, M. T. (2023). A systematic review of Black People coping with racism: Approaches, analysis, and empowerment. *Perspectives on Psychological Science*, 18(2), 392–415. <https://doi.org/10.1177/17456916221100509>
- Jaspal, R., da Silva Lopes, B. C., & Breakwell, G. M. (2021). British national identity and life satisfaction in ethnic minorities in the United Kingdom. *National Identities*, 23(5), 455–472. <https://doi.org/10.1080/14608944.2020.1822793>
- Kaholokula, J. K. A., Antonio, M. C., Ing, C. K. T., Hermosura, A., Hall, K. E., Knight, R., & Willis, T. A. (2017). The effects of perceived racism on psychological distress mediated by venting and disengagement coping in Native Hawaiians. *BMC Psychology*, 5, 1–10. <https://doi.org/10.1186/s40359-017-0171-6>
- Kirkinis, K., Pieterse, A. L., Martin, C., Agiliga, A., & Brownell, A. (2021). Racism, racial discrimination, and trauma: A systematic review of the social science literature. *Ethnicity Health*, 26(3), 392–412. <https://doi.org/10.1080/13557858.2018.1514453>
- Koff, E., & Sangani, P. (1997). Effects of coping style and negative body image on eating disturbance. *International Journal of Eating Disorders*, 22(1), 51–56. [https://doi.org/10.1002/\(SICI\)1098-108X\(199707\)22:1<51::AID-EAT6>3.0.CO;2-1](https://doi.org/10.1002/(SICI)1098-108X(199707)22:1<51::AID-EAT6>3.0.CO;2-1)
- Krägeloh, C. U. (2011). A systematic review of studies using the brief COPE: Religious coping in factor analyses. *Religions*, 2(3), 216–246. <https://doi.org/10.3390/rel2030216>
- Krieger, N., Smith, K., Naishadham, D., Hartman, C., & Barbeau, E. M. (2005). Experiences of discrimination: Validity and reliability of a self-report measure for population health research on racism and health. *Social Science Medicine*, 61(7), 1576–1596. <https://doi.org/10.1016/j.socscimed.2005.03.006>
- Landor, A. M., Ramseyer Winter, V. L., Thurston, I. B., Chan, J., Craddock, N., Ladd, B. A., Tylka, T. L., Swami, V., Watson, L. B., & Choukas-Bradley, S. (2024). The Sociocultural-Intersectional Body Image (SIBI) framework: Understanding the impact of white supremacy in body image research and practice. *Body Image*, 48, Article 101674. <https://doi.org/10.1016/j.bodyim.2023.101674>
- Landrine, H., & Klonoff, E. A. (1996). The Schedule of Racist Events: A measure of racial discrimination and a study of its negative physical and mental health consequences. *Journal of Black Psychology*, 22(2), 144–168. <https://doi.org/10.1177/0095798496022002>
- Lazarus, R., & Folkman, S. (1984). *Stress, appraisal and coping*. New York, NY: Springer.
- Le, T. P., Iwamoto, D. K., & Souliard, Z. A. (2022). Body positivity for Asian Americans: Development and evaluation of the pride in Asian American Appearance Scale. *Journal of Counseling Psychology*, 69(5), 614–629. <https://doi.org/10.1037/cou0000622>
- Linarón, J., McClure, Z., Tylka, T. L., & Fuller-Tyszkiewicz, M. (2022). Body appreciation and its psychological correlates: A systematic review and meta-analysis. *Body Image*, 42, 287–296. <https://doi.org/10.1016/j.bodyim.2022.07.003>
- Litman, J. A. (2006). The COPE inventory: Dimensionality and relationships with approach-and avoidance-motives and positive and negative traits. *Personality and Individual Differences*, 41(2), 273–284. <https://doi.org/10.1016/j.paid.2005.11.032>
- Mason, T. B., Lewis, R. J., & Heron, K. E. (2018). Disordered eating and body image concerns among sexual minority women: A systematic review and testable model. *Psychology of Sexual Orientation and Gender Diversity*, 5, 397–422. <https://doi.org/10.1037/sgd0000293>
- Mason, T. B., & Lewis, R. J. (2016). Minority stress, body shame, and binge eating among lesbian women: Social anxiety as a linking mechanism. *Psychology of Women Quarterly*, 40, 428–440. <https://doi.org/10.1177/036184316635529>
- Mason, B., Mozdierz, P., Wang, S., & Smith, K. E. (2021). Discrimination and eating disorder psychopathology: A meta-analysis. *Behavior Therapy*, 52(2), 406–417. <https://doi.org/10.1016/j.beth.2020.05.003>
- Mason, B., Smith, K. E., & Lavender, J. M. (2019). Stigma control model of dysregulated eating: A momentary maintenance model of dysregulated eating among marginalized/stigmatized individuals. *Appetite*, 132, 67–72. <https://doi.org/10.1016/j.appet.2018.09.017>
- Michaels, E., Thomas, M., Reeves, A., Price, M., Hasson, R., Chae, D., & Allen, A. (2019). Coding the Everyday Discrimination Scale: Implications for exposure assessment and associations with hypertension and depression among a cross section of mid-life African American women. *Journal of Epidemiology and Community Health*, 73(6), 577–584. <https://doi.org/10.1136/jech-2018-211230>
- Modica, C. A., & Markley, A. E. (2024). Reciprocal longitudinal associations between positive body image and wellbeing among early-adult women. *Body Image*, 51, 101770. <https://doi.org/10.1016/j.bodyim.2024.101770>
- Moos, R. H., & Holahan, C. J. (2003). Dispositional and contextual perspectives on coping: Toward an integrative framework. *Journal of Clinical Psychology*, 59(12), 1387–1403. <https://doi.org/10.1002/jclp.10229>
- Muthén, L. K., & Muthén, B. (2024). *Mplus user's guide, Version 8.11*. Muthén & Muthén. National Academies of Sciences, Engineering, and Medicine. (2023). *Using population descriptors in genetics and genomics research: A new framework for an evolving field*. Washington, DC: The National Academies Press. <https://doi.org/10.17226/26902>
- Palan, S., & Schitter, C. (2018). Prolific.ac – A subject pool for online experiments. *Journal of Behavioral and Experimental Finance*, 17, 22–27. <https://doi.org/10.1016/j.jbef.2017.12.004>
- Paradies, Y., Ben, J., Denson, N., Elias, A., Priest, N., Pieterse, A., Gupta, A., Kelaher, M., & Gee, G. (2015). Racism as a determinant of health: A systematic review and meta-

- analysis. *PLoS ONE*, 10(9), Article e0138511. <https://doi.org/10.1371/journal.pone.0138511>
- Pascoe, E. A., & Richman, L. S. (2009). Perceived discrimination and health: A meta-analysis review. *Psychological Bulletin*, 135(4), 531–554. <https://doi.org/10.1037/a0016059>
- Pieterse, A. L., & Powell, S. (2016). A theoretical overview of the impact of racism on people of color. In A. Alvarez, C. Liang, & H. A. Neville (Eds.), *Contextualizing the costs of racism* (pp. 11–30). American Psychological Association.
- R Development Core Team. (2021). R: A language and environment for statistical computing. R Foundation for Statistical Computing.
- Ramseyer Winter, V., Danforth, L. K., Landor, A., & Pevehouse-Pfeiffer, D. (2019). Toward an understanding of racial and ethnic diversity in body image among women. *Social Work Research*, 43(2), 69–80. <https://doi.org/10.1093/swr/svy033>
- Reskin, B. (2012). The race discrimination system. *Annual Review of Sociology*, 38, 17–35. <https://doi.org/10.1146/annurev-soc-071811-145508>
- Rosseel, Y. (2012). lavaan: An R package for structural equation modeling. *Journal of Statistical Software*, 48(2), 1–36. <https://doi.org/10.18637/jss.v048.i02>
- Schmitt, M. T., Branscombe, N. R., Postmes, T., & Garcia, A. (2014). The consequences of perceived discrimination for psychological well-being: a meta-analytic review. *Psychological Bulletin*, 140(4), 921–948. <https://doi.org/10.1037/a0035754>
- Skinner, E. A., Edge, K., Altman, J., & Sherwood, H. (2003). Searching for the structure of coping: a review and critique of category systems for classifying ways of coping. *Psychological Bulletin*, 129(2), 216. <https://doi.org/10.1037/0033-2909.129.2.216>
- Smith-Jackson, T., Reel, J. J., & Thackeray, R. (2011). Coping with “bad body image days”: Strategies from first-year young adult college women. *Body Image*, 8(4), 335–342. <https://doi.org/10.1016/j.bodyim.2011.05.002>
- Suyemoto, K. L., Abdullah, T., Godon-Decoteau, D., Tahirkheli, N. N., Arbid, N., & Frye, A. A. (2022). Development of the Resistance and Empowerment Against Racism (REAR) Scale. *Cultural Diversity and Ethnic Minority Psychology*, 28(1), 58–71. <https://doi.org/10.1037/cdp0000353>
- Swami, V., & Barron, D. (2019). Translation and validation of body image instruments: Challenges, good practice guidelines, and reporting recommendations for test adaptation. *Body Image*, 31, 204–220. <https://doi.org/10.1016/j.bodyim.2018.08.014>
- Swami, V., Tran, U. S., Stieger, S., Aavik, T., Abdollahpour Ranjbar, H., Adebayo, S. O., Afhami, R., Ahmed, O., Aimé, A., Akel, M., Al Halbusi, H., Alexias, G., Ali, K. F., Alpdal, N., Alsalhani, A. B., Álvares-Solas, S., Amaral, A. C. S., Andrianto, S., Aspdén, T., & Voracek, M. (2023). Body appreciation around the world: Measurement invariance of the Body Appreciation Scale-2 (BAS-2) across 65 nations, 40 languages, gender identities, and age. *Body Image*, 46, 449–466. <https://doi.org/10.1016/j.bodyim.2023.07.010>
- Tamres, L. K., Janicki, D., & Helgeson, V. S. (2002). Sex differences in coping behavior: A meta-analytic review and an examination of relative coping. *Personality and Social Psychology Review*, 6(1), 2–30. https://doi.org/10.1207/S15327957PSPR0601_1
- Thomas, A. J., Witherspoon, K. M., & Speight, S. L. (2008). Gendered racism, psychological distress, and coping styles of African American women. *Cultural Diversity and Ethnic Minority Psychology*, 14(4), 307–314. <https://doi.org/10.1037/1099-9809.14.4.307>
- Tiggemann, M., & Hage, K. (2019). Religion and spirituality: Pathways to positive body image. *Body Image*, 28, 135–141. <https://doi.org/10.1016/j.bodyim.2019.01.004>
- Tylka, T. L. (2018). Overview of the field of positive body image. In E. A. Daniels, M. M. Gillen, & C. H. Markey (Eds.), *Body positive: Understanding and improving body image in science and practice* (pp. 6–33). New York, NY: Cambridge University Press.
- Tylka, T. L., & Wood-Barcalow, N. L. (2015a). What is and what is not positive body image? Conceptual foundations and construct definition. *Body Image*, 14, 118–129. <https://doi.org/10.1016/j.bodyim.2015.04.001>
- Tylka, T. L., & Wood-Barcalow, N. L. (2015b). The Body Appreciation Scale-2: Item refinement and psychometric evaluation. *Body Image*, 12, 53–67. <https://doi.org/10.1016/j.bodyim.2014.09.006>
- Vassilière, C. T., Holahan, C. J., & Holahan, C. K. (2016). Race, perceived discrimination, and emotion-focused coping. *Journal of Community Psychology*, 44(4), 524–530. <https://doi.org/10.1002/jcop.21776>
- Velez, B. L., Campos, I. D., & Moradi, B. (2015). Relations of sexual objectification and racist discrimination with Latina women's body image and mental health. *The Counseling Psychologist*, 43(6), 906–935. <https://doi.org/10.1177/0011000015591287>
- Wallace, S., Nazroo, J., & Bécaries, L. (2016). Cumulative effect of racial discrimination on the mental health of ethnic minorities in the United Kingdom. *American Journal of Public Health*, 106(7), 1294–1300. <https://doi.org/10.2105/AJPH.2016.303121>
- Wang, J., Zamar, R., Marazzi, A., Yohai, V., Salibian-Barrera, M., Maronna, R., Zivot, E., Rocke, D., Martin, D., & Konis, K. (2013). Robust: Insightful Robust library. *R Package Version*, 0, 4–15. (<http://CRAN.R-project.org/package=robust>).
- Williams, D. R., Lawrence, J. A., Davis, B. A., & Vu, C. (2019). Understanding how discrimination can affect health. *Health Services Research*, 54, 1374–1388.
- Williams, D. R., Yu, Y., Jackson, J. S., & Anderson, N. B. (1997). Racial differences in physical and mental health: Socio-economic status, stress and discrimination. *Journal of Health Psychology*, 2(3), 335–351. <https://doi.org/10.1177/135910539700200305>
- Wood-Barcalow, N. L., Tylka, T. L., & Augustus-Horvath, C. L. (2010). But I like my body”: Positive body image characteristics and a holistic model for young-adult women. *Body image*, 7(2), 106–116. <https://doi.org/10.1016/j.bodyim.2010.01.001>